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PACU Discharge Safety Interventions In Adults

A Rapid Scoping Review and Evidence Map

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Scan for full supplement
Search strategy • included studies • references • evidence map
Link: **linkto.click/du**

Background

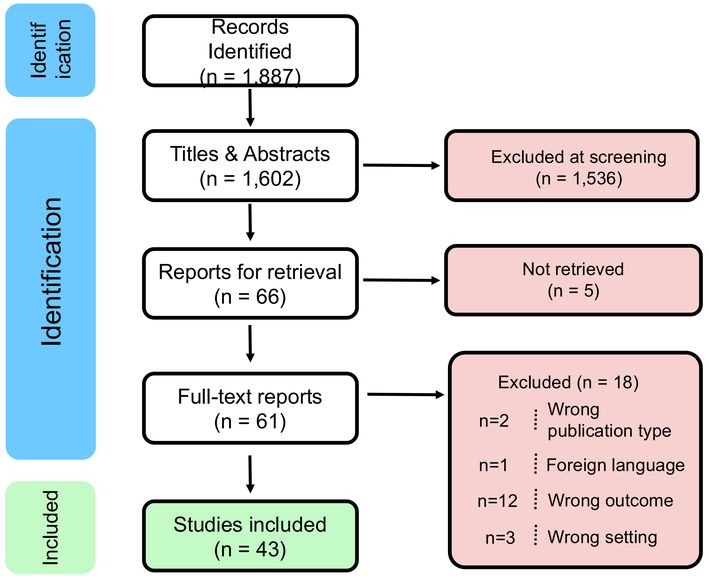
- Transition from continuous monitoring to lower-acuity care
- Early deterioration can occur after PACU discharge
- Wide variation in discharge tools and workflows

Objectives

- Map adult PACU discharge safety interventions
- Characterize outcome reporting
- Identify evidence gaps

Methods

- Rapid scoping review (PRISMA-ScR)
- PubMed/MEDLINE (2010–2026)
- Single reviewer and spot check



Results

Evidence Map of PACU Discharge Interventions and Outcomes

Intervention Category	Safety Events	Escalation / Rescue	Process	Throughput	Post-discharge Utilization
Decision Support	12	8	7	6	5
Discharge Checklist / Criteria	6	1	4	7	4
Handoff / Transition	3	1	5	0	0
Nurse-led Protocol / Workflow	12	11	12	10	8
TOTALS (by Outcome)	33	21	28	23	14

Deep Green: High Count (10+)	Light Green: Medium Count (6-9)	Yellow: Low Count (3-5)	Very Low/No Count (0-1)

Note: Studies may contribute to multiple outcome domains

Key Findings

- Evidence is clustered in workflows and safety detection
- Emphasis on safety and process measures more than escalation or downstream utilization
- Outcome reporting varies across intervention types
- Most studies are observational

Conclusions & Implications

- Evidence does not consistently evaluate clinically meaningful outcomes
- Outcome definitions are not standardized across studies
- Limited evidence exists to guide PACU discharge decision-making
- More prospective and workflow-integrated studies are needed to evaluate discharge decision strategies